

Key words: genodermatoses; growth hormone; melanoma; nevi; sex chromosome abnormality; Turner syndrome.

Correspondence to: Megan N. Rogge, MD, Department of Dermatology, The University of Texas Health Science Center at Houston, 6500 West Loop South, Ste 130, Bellaire, TX 77401

E-mail: megan.rogge@uth.tmc.edu

Conflicts of interest

None disclosed.

REFERENCES

- Lowenstein EJ, Kim KH, Glick SA. Turner's syndrome in dermatology. *J Am Acad Dermatol*. 2004;50(5):767-776. <https://doi.org/10.1016/j.jaad.2003.07.031>
- Swerdlow AJ, English J, MacKie RM, et al. Benign melanocytic naevi as a risk factor for malignant melanoma. *BMJ*. 1986; 292(6535):1555-1559. <https://doi.org/10.1136/bmj.292.6535.1555>
- Ji J, Zöller B, Sundquist J, Sundquist K. Risk of solid tumors and hematological malignancy in persons with Turner and Klinefelter syndromes: a national cohort study. *Int J Cancer*. 2016; 139(4):754-758. <https://doi.org/10.1002/ijc.30126>
- Piérard GE, Piérard-Franchimont C, Nikkels A, Nikkels-Tassoudji N, Arrese JE, Bourguignon JP. Naevocyte triggering by recombinant human growth hormone. *J Pathol*. 1996;180(1):74-79. [https://doi.org/10.1002/\(SICI\)1096-9896\(199609\)180:1<74::AID-PATH595>3.0.CO;2-A](https://doi.org/10.1002/(SICI)1096-9896(199609)180:1<74::AID-PATH595>3.0.CO;2-A)
- Wyatt D. Melanocytic nevi in children treated with growth hormone. *Pediatrics*. 1999;104(4 Pt 2):1045-1050.

<https://doi.org/10.1016/j.jaad.2024.06.034>

No significant disparities in nonmelanoma skin cancer between Latino and White cohorts in Miami, Florida: A single-center retrospective analysis



To the Editor: A recent study examining differences in post-Mohs micrographic surgery (MMS) defect sizes, using it as an adjunct to assess carcinoma severity, between Latino and non-Latino White patients in Los Angeles found that Latinos experience increased disease burden with nonmelanoma skin cancer.¹ To further investigate this disparity, we performed an institutional review board-approved retrospective study comparing pre-MMS tumor size, number of stages taken, and postsurgical defect size between Latino and non-Latino White patients seen at the University of Miami Miller School of Medicine who underwent MMS to assess for disparities. Although we similarly compare Latino with non-Latino White patients, highlighting the demographic differences between Miami and Los Angeles Latinos is essential. Latinos account for

Table I. Demographic data of Latino and Non-Latino White patients undergoing Mohs micrographic surgery in the study

Demographics	Latino	Non-Latino White
N	344	466
Mean age (SD)	68.36 (13.59)	70.86 (11.54)
18-39, n (%)	10 (2.91)	8 (1.71)
40-60, n (%)	76 (22.09)	58 (12.45)
61-74, n (%)	131 (38.08)	191 (40.99)
75+, n (%)	127 (36.92)	209 (44.85)
Insurance status		
HMO (%)	128 (37.21)	75 (16.09)
Medicaid (%)	32 (9.30)	7 (1.50)
Medicare (%)	136 (39.54)	249 (53.43)
PPO (%)	26 (7.56)	37 (7.94)
Uninsured/self-pay (%)	5 (1.45)	5 (1.07)
VA (%)	17 (4.94)	93 (19.95)
Gender		
Male (%)	243 (70.64)	355 (76.18)
Female (%)	101 (29.36)	111 (23.82)

HMO, Health maintenance organization; PPO, preferred provider organization; SD, standard deviation; VA, veterans affairs.

46.9% of the Los Angeles population, and most are of Mexican descent. Miami, by contrast, has a Latino population mostly of Caribbean descent and accounts for 70.2% of the city's population.² The Latino ethnicity encompasses many different populations that may have variability in Fitzpatrick skin type and European ancestry, as well as differing attitudes regarding photoprotection and recognition of suspicious lesions, prompting a visit to a dermatologist, among other cultural differences and barriers to care.

Our study included 819 patients seen between September 1st, 2022, and December 31st, 2023. The data we collected from each patient included race/ethnicity, age, gender, insurance status, initial lesion size, number of stages taken, post-MMS defect size, and type of keratinocyte carcinoma. Demographic data is shown in Table I. Asian, Black, and Native American groups were not included in our analysis as there were too few patients.

Table II compares the mean initial lesion size, the mean number of stages taken, and the mean postsurgical defect size. A two-tailed t-test was performed to assess for statistical significance, and *P* values < .05 were considered significant. There was no statistically significant difference in any of the metrics for basal cell carcinoma, squamous cell carcinoma, and squamous cell carcinoma in situ between Latino and non-Latino White patients, contrasting with the previous study performed in Los Angeles.

Table II. Comparison of mean pre-surgery lesion size, stages taken, and post-surgical defect size between Latino and non-Latino White patients

Keratinocyte carcinoma type	Latino patients	Non-Latino White patients	P-value
BCC (n)	231	303	
Mean pre-surgery lesion size (cm ² ± SD)	1.36 ± 2.68	1.32 ± 2.76	.87
Mean stages taken (n)	1.79 ± 0.87	1.75 ± 0.86	.60
Mean post-surgical defect size (cm ² ± SD)	3.13 ± 5.28	3.08 ± 5.54	.92
SCC (n)	77	106	
Mean pre-surgery lesion size (cm ² ± SD)	2.08 ± 3.64	1.76 ± 1.66	.43
Mean stages taken (n)	1.79 ± 0.81	1.59 ± 0.68	.07
Mean post-surgical defect size (cm ² ± SD)	4.20 ± 6.22	3.47 ± 3.30	.31
SCCIS (n)	36	57	
Mean pre-surgery lesion size (cm ² ± SD)	1.59 ± 1.20	1.45 ± 1.26	.60
Mean stages taken (n)	1.47 ± 0.69	1.68 ± 0.63	.14
Mean post-surgical defect size (cm ² ± SD)	3.49 ± 2.92	2.95 ± 2.22	.32

BCC, Basal cell carcinoma; SCC, squamous cell carcinoma; SCCIS, squamous cell carcinoma in situ.

While it is difficult to pinpoint the reasons for our findings, which are likely multifactorial, we highlight that most of the Miami population is of Latino descent, which likely positively impacts patients of this background in navigating the healthcare system and accessing timely dermatologic care. Furthermore, while only 6% of California physicians are Latinos, 18% of physicians in Florida are Latino.^{3,4} Some studies have shown that racial and ethnic concordance between physicians and Black/Latino patients may correlate with greater trust in healthcare.⁵ Our findings may be in part due to an increased percentage of minority physicians treating this patient population, underscoring the importance of minority physicians in addressing healthcare disparities and the importance of increasing the representation of underrepresented groups in medicine, particularly in dermatology, which lags other fields in diversity. A limitation of our study is that we did not have data describing the Fitzpatrick skin type of each patient encountered. Further studies examining disparities and potential solutions in dermatology are needed to increase health equity.

Philippe Jean-Pierre, BS, Ryan J. Scheinkman, MEng, and Keyvan Nouri, MD, MBA

From the Phillip Frost Department of Dermatology and Cutaneous Surgery, University of Miami, Miami, Florida.

Funding sources: None.

Patient consent: Not applicable.

IRB approval status: The University of Miami Institutional Review Board approved this study.

Key words: disparities; diversity; Latino; mohs surgery; non-melanoma; skin cancer; skin of color; surgical dermatology.

Correspondence to: Philippe Jean-Pierre, BS, Phillip Frost Department of Dermatology and Cutaneous Surgery, University of Miami, 1150 NW 14th St, Suite 500, Miami, FL 33136

E-mail: philanator@med.miami.edu

Conflicts of interest

None disclosed.

REFERENCES

- Blumenthal LY, Arzeno J, Syder N, et al. Disparities in nonmelanoma skin cancer in Hispanic/Latino patients based on Mohs micrographic surgery defect size: a multicenter retrospective study. *J Am Acad Dermatol*. 2022;86(2):353-358. <https://doi.org/10.1016/j.jaad.2021.08.052>
- United States Census Bureau. Accessed February 12, 2024. <https://www.census.gov/data.html>
- Balderas-Medina Anaya Y, Hsu P, Greenwood SS, Hayes-Bautista D. Improvement needed in Latina physician representation: Implications for medical education, training and policy. UCLA Latino Policy & Politics Institute. Accessed February 12, 2024. <https://latino.ucla.edu/research/improvement-needed-in-latina-physician-representation/>
- 2023 Florida physician workforce annual report. Accessed February 12, 2024. <https://www.floridahealth.gov/provider-and-partner-resources/community-health-workers/HealthResourcesandAccess/physician-workforce-development-and-recruitment/2023DOHPhysicianWorkforceAnnualReport-FINAL1.pdf>
- Jetty A, Jabbarpour Y, Pollack J, Huerto R, Woo S, Petterson S. Patient-physician racial concordance associated with improved healthcare use and lower healthcare expenditures in minority populations. *J Racial and Ethnic Health Disparities*. 2022;9(1):68-81. <https://doi.org/10.1007/s40615-020-00930-4>

<https://doi.org/10.1016/j.jaad.2024.06.035>